PharmEasy Labs

NAME	: ADITI GUPTA(20Y/F)	**HOME COLLECTION :**	
REF. BY	: SELF	A 104 MOONLIGHT COOPERATIVE HOUSING SOCIETY UNITECH WEST END VIRAR WEST KARGIL NAGAR VIRAR	
TEST ASKED	: MASTER CHECKUP WITH CANCER AND ARTHRITIS SCREENING (FEMALE) - NEW		

TEST NAME	TECHNOLOGY	VALUE	UNITS
CA-125	C.L.I.A	29.7	U/mL

Bio. Ref. Interval. :-

Less than 30.2 U/ml

Clinical Significance:

CA-125 is used to monitor therapy during treatment for Ovarian Cancer. CA125 is also to detect or monitor whether there is a recurrence of cancer or malignancy after surgical removal of tumor or radiation therapy or chemotherapy (antineoplastic drugs). This test is sometimes used to follow High-Risk women who have a family history of Ovarian Cancer. CA-125 may normally be increased in early pregnancy and during menstruation. It can also be increased in diseases such as Pelvic Inflammatory Disease or Endometriosis and sometimes in Hepatitis and Cirrhosis of the liver.

Specifications:

Precision: Intra Assay (%CV): 4.3 %, Inter Assay (%CV): 2.5%; Sensitivity: 2.0 U/ml

Kit Validation References:

Mackey SE, Creasman WT. Ovarian Cancer Screening. J. Clin Oncol 1995; 13(3); 783 - 93.

Please correlate with clinical conditions.

Method:- TWO SITE SANDWICH IMMUNOASSAY

Sample Collected on (SCT)	: 15 Nov 2023 09:22
Sample Received on (SRT)	: 15 Nov 2023 11:41
Report Released on (RRT)	: 15 Nov 2023 17:41
Sample Type	: SERUM
Labcode	: 1511067185/DG007
Barcode	: BO865015

Dr Vitthal Kendre MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd

PharmEasy Labs
Registered Address: D-37/3, TTC MIDC, Turbhe, Navi Mumbai, 400703
Lab Address: D-37/1, TTC MIDC, Turbhe, Navi Mumbai, 400703
7022 000 900

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	PHOTOMETRY	113	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**39**	**mg/dL**	**40-60**
HDL / LDL RATIO	CALCULATED	0.53	Ratio	> 0.40
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	73	mg/dL	< 100
TRIG / HDL RATIO	CALCULATED	1.63	Ratio	< 3.12
TRIGLYCERIDES	PHOTOMETRY	64	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	**CALCULATED**	**2.9**	**Ratio**	**3 - 5**
LDL / HDL RATIO	CALCULATED	1.9	Ratio	1.5-3.5
NON-HDL CHOLESTEROL	CALCULATED	73.9	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	12.72	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
HD/LD - Derived from HDL and LDL values.
LDL - Direct Measure
TRI/H - Derived from TRIG and HDL Values
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
LDL/ - Derived from serum HDL and LDL Values
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	136	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- CKD-EPI Creatinine Equation

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